



Motivation (Motivational Interviewing: MI)

Dr Shanthi Vanka

REFERENCE: Chapter 4- Motivational Interviewing (MI) and its basic tools HEALTH BEHAVIOR CHANGE IN THE DENTAL PRACTICE edited by Christoph A. Ramseier and Jean E. Suvan

Lecture learning outcomes

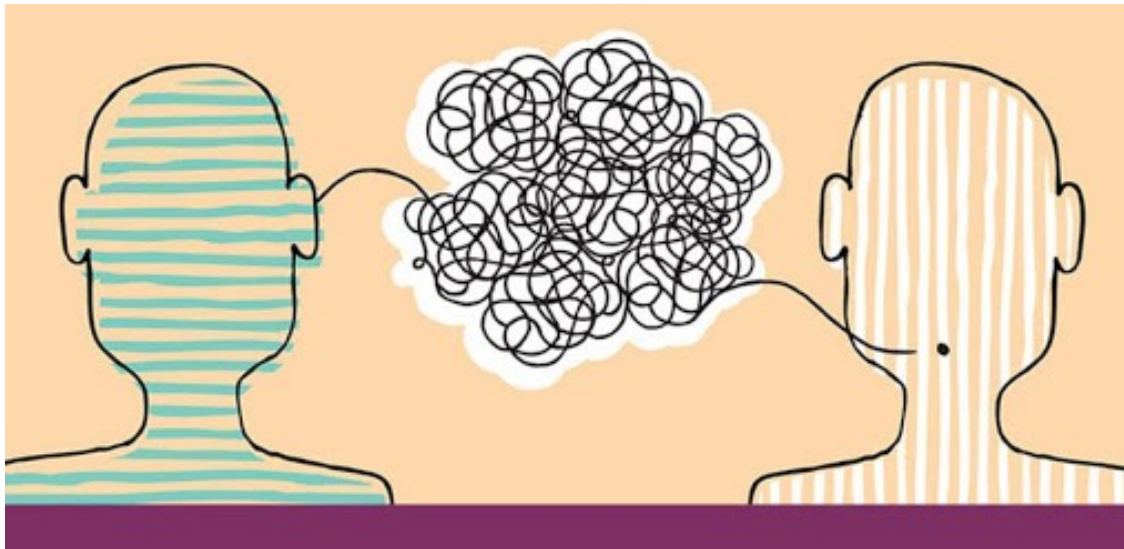
- ▶ Describe Motivational interviewing and its basic tools
- ▶ List various principles in implementing the spirit of motivational interviewing

Outline

- ▶ What is Motivational Interviewing (MI)
- ▶ What triggers behaviour change
- ▶ Motivation and clinician pt relationship- Exploring the gaps for behaviour change
- ▶ Spirit of MI
- ▶ Principles of MI
- ▶ Process of MI
- ▶ Tools for building motivation
- ▶ Elicit talk
- ▶ Respond to change talk
- ▶ Summary

What is Motivational Interviewing (MI)?

- ▶ Is a **method of communication** rather than a set of techniques
- ▶ **Style or method of counselling patients**



MI Approach

- ▶ **Non-confrontational** and **empathy** approach
- ▶ A **strong alliance** or bond between **counsellor** and **patient**



Research evidence for MI

- ▶ MI is highly efficient compared to other methods with as little as **15 minutes** of **interaction** shown to be **effective** in the majority of studies.



What triggers behavior change?

- ▶ **Educational approach** alone leads to **poor outcomes**
- ▶ **Increased patient knowledge** will **translate** into **behavior change**.



MI Approach (What is the basis for individuals to be motivated?)

- ▶ MI is **based** on the **assumption** that individuals are motivated to **change** when change is **connected** to something they **value**.



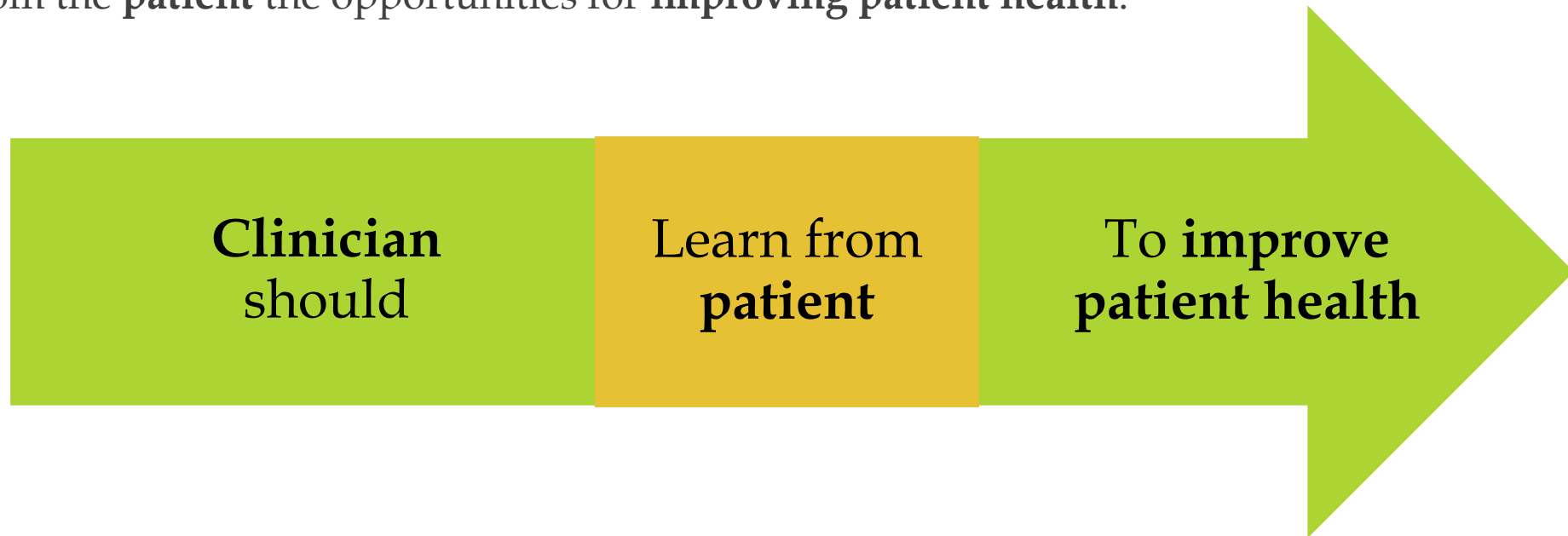
The goal of MI

- Is the willingness to **explore** both sides of a **patient's ambivalence** (state of having mixed feelings) to **help him or her** consider **what** he or she **truly values**.



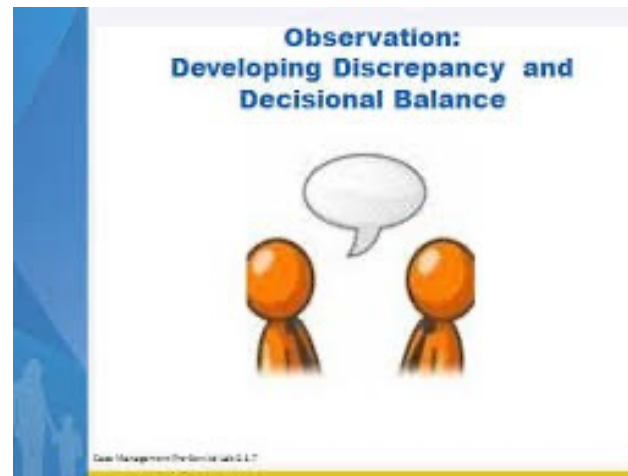
Motivation and the clinician - patient relationship

- ▶ Clinicians may be more effective when they see themselves as needing to **learn** from the **patient** the opportunities for **improving patient health**.



Developing discrepancy

- ▶ MI aims to **avoid lecturing** or **persuading**.
- ▶ It involves **exploring** with patients the **gap** between their **goals** or **values**.



People who are smokers actually would prefer to be non - smokers

- ▶ During this process, the clinician also **attempts to elicit “ change talk ”** or statements that are consistent with or in the direction of making a change for example:
- ▶ Ex: “ **Even though I’m not really doing it, I would like to stop smoking. ”**



Earlier stage

- ▶ In the earliest stages of considering a change, so - called change talk often takes the form of simply **recognizing or acknowledging there is a problem**, again
- ▶ Ex: “ **I know I should brush more regularly.** ”



Later stages

- ▶ In the later stages it takes the form of **expressing optimism or the intention to change** and, therefore, the patient may say:
- ▶ Ex: “ I ’ll definitely start brushing and flossing twice a day. ”



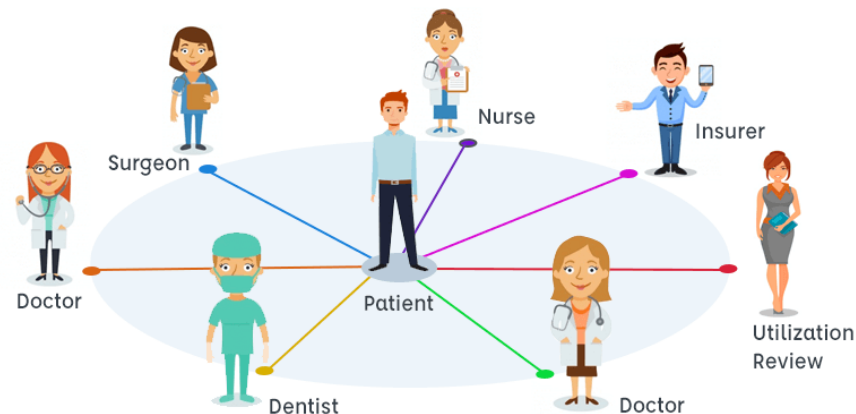
Key issue of MI

- ▶ The **clinician (Dentist)** has the **ability to influence the relationship** between **clinician (Dentist)** and **patient** to either **foster resistance** or **develop discrepancy** and **elicit change talk**.



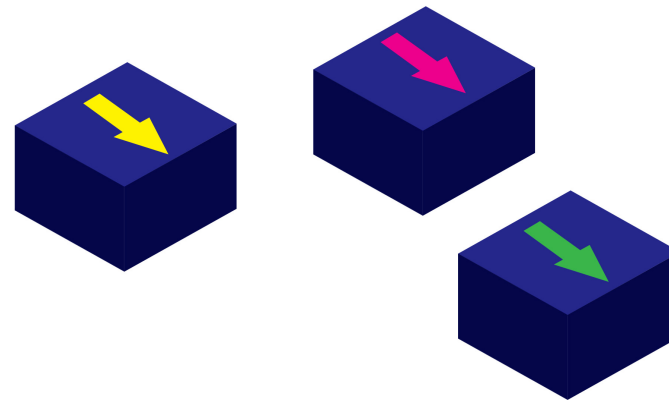
Definition of MI

- ▶ MI is client or **patient - centered** in that the clinician attempts to work from the **patient's perspective**.
- ▶ **Intrinsic or "internal"** reasons for change tend to be **associated** with **long - term behavior change**

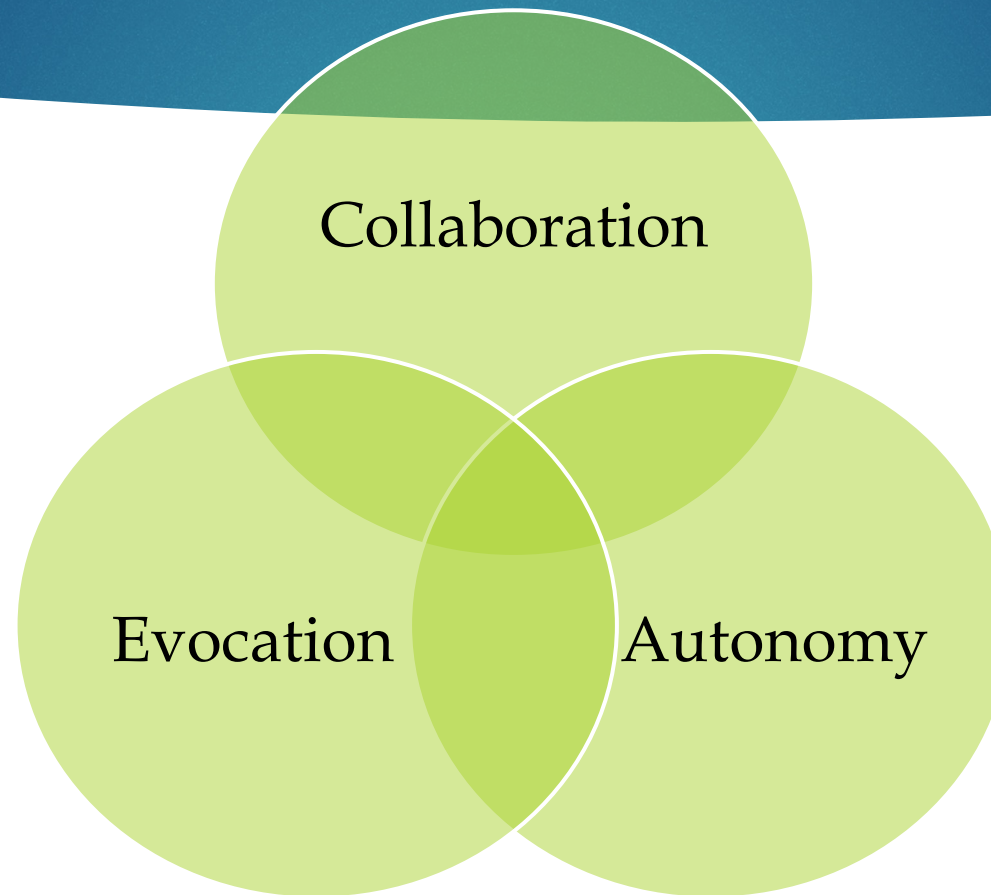


Patient centered approach on MI

- ▶ **Goal of exploring and attempting to facilitate change in a particular direction** (e.g., to brush twice daily, to floss more frequently, to avoid sugary foods, etc.).



SPIRIT of MI



Collaboration

- ▶ Refers to a **style of working with the patient**
- ▶ The **clinician fosters a partnership** with **the patient** rather than assuming an expert role in which the patient is a recipient.



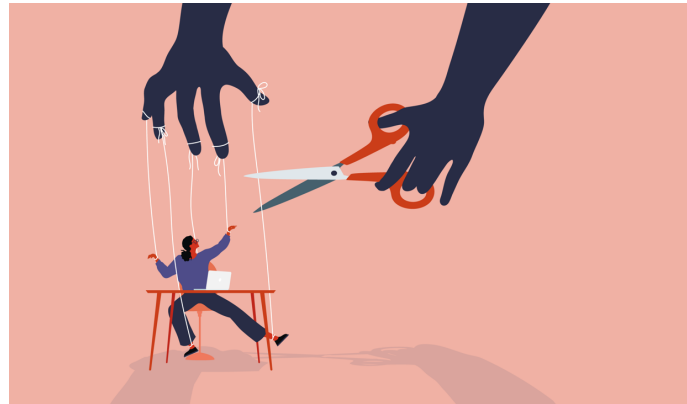
Evocation

- ▶ A style that emphasizes eliciting the motivation from “ within ” the patient rather than trying to impose motivation from the “ outside. ”
- ▶ **Motivation** should be **intrinsic**

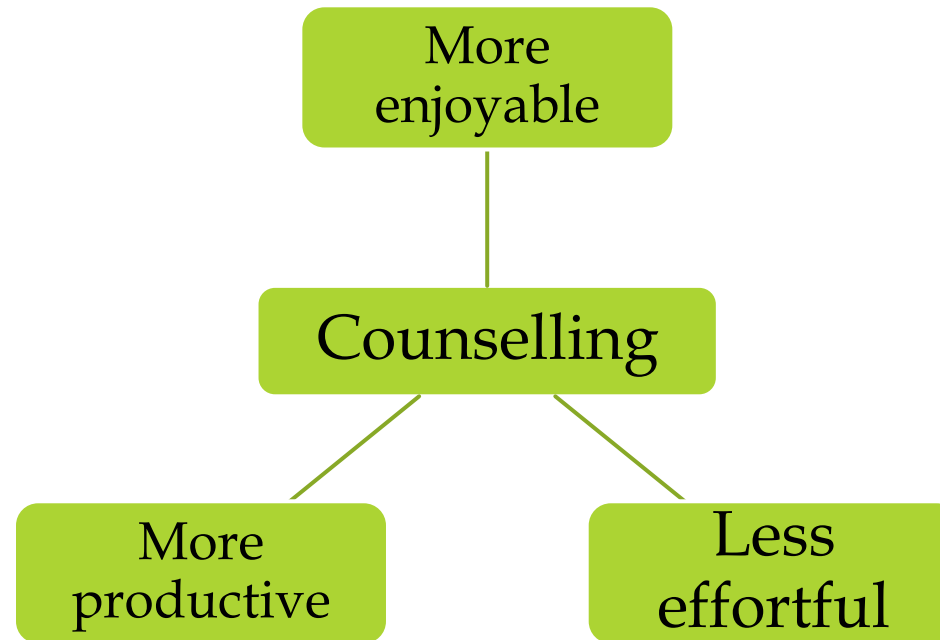


Autonomy

- ▶ **Freedom of patients to make their own choices.**
- ▶ The responsibility for change is not in the hands of the clinician.
- ▶ The **final decision on what to do rests with the patient.**

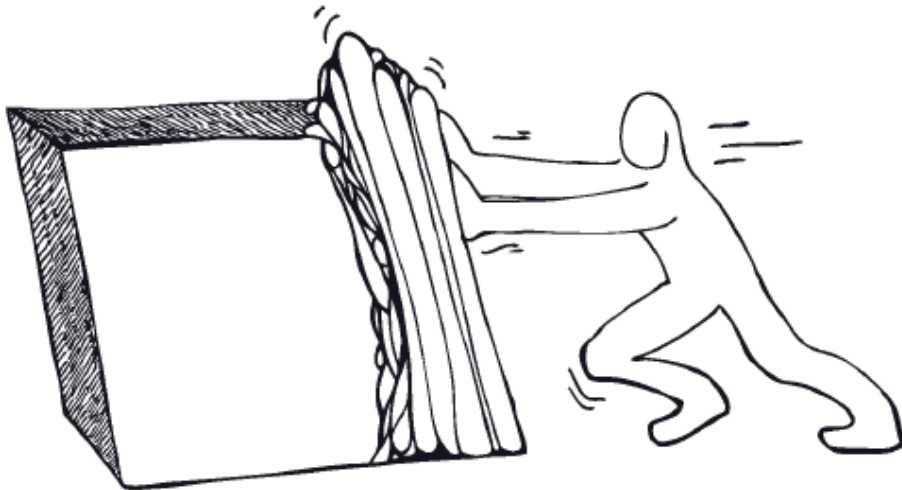


Implementation of the 3 elements of MI



How do MI elements bring about a change in a patient behavior?

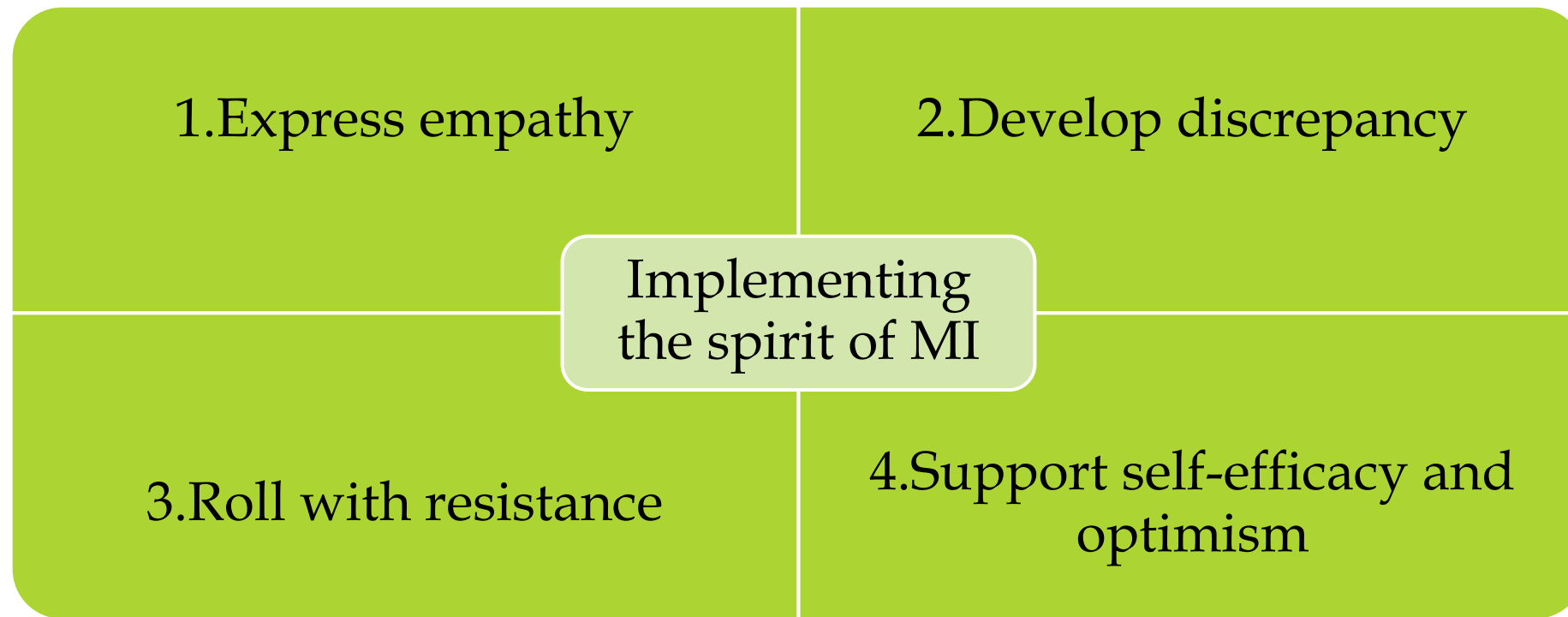
Pushing for change



Working with the patient



Major principles



Express empathy

- ▶ “ **Empathy** ” refers to understanding the perspective of another person
- ▶ This is achieved when **communicating acceptance** and using **reflective listening** skills.



Develop discrepancy

- ▶ **Exploring how current behavior fits in or could be changed to achieve the goals and values.**
- ▶ For example, a patient may have a goal of being a good parent. Eating in a healthy way and modelling good oral hygiene practices may be an important way to live out the value of being a good parent.



Roll with resistance

- ▶ Clinician **rolls with resistance** and **avoids arguing**
- ▶ Pushing back, persuading, or arguing tends to increase resistance, so the MI clinician instead attempts to roll with resistance

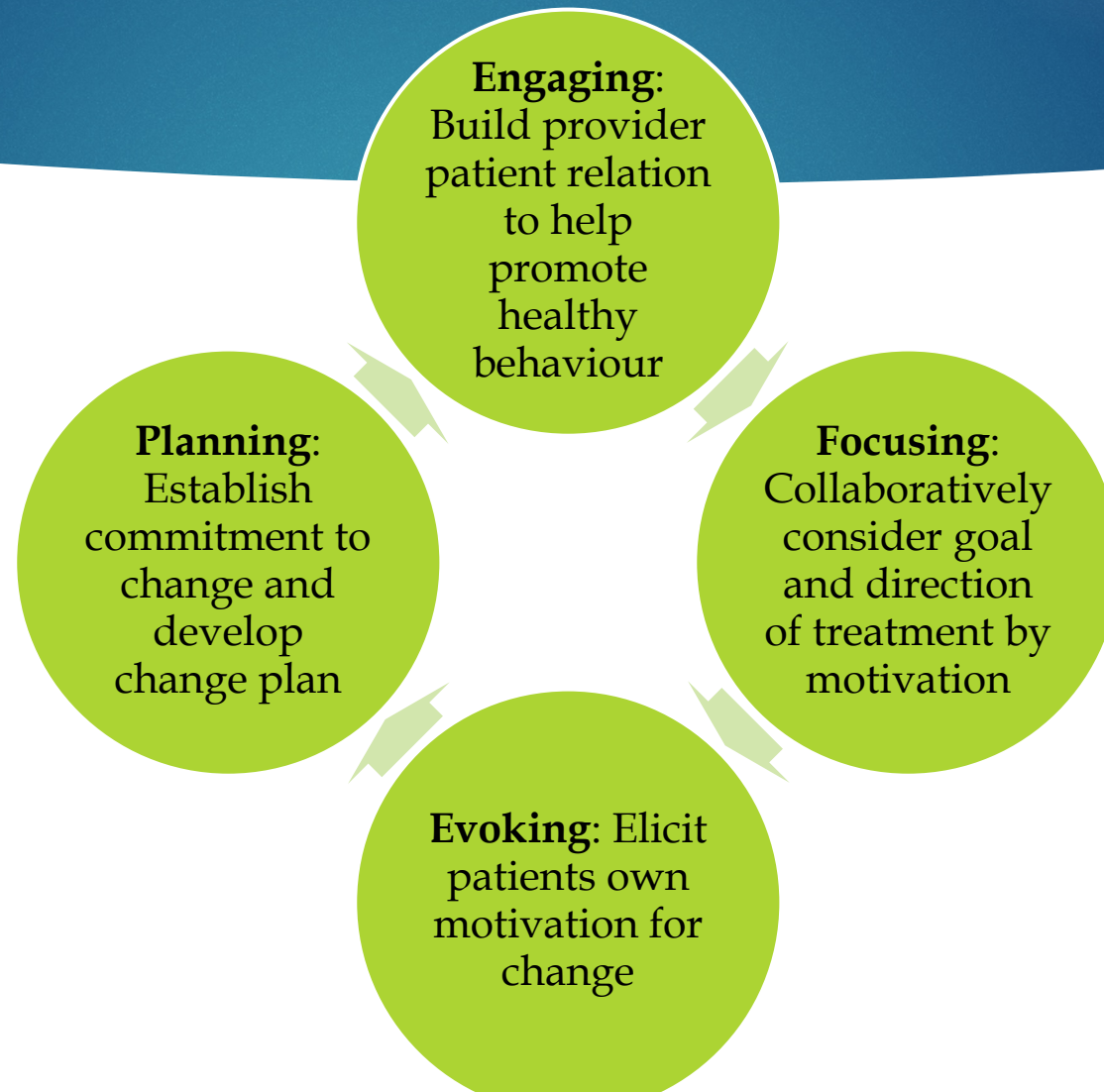


Support self-efficacy

- ▶ Support self - efficacy (or confidence) and optimism for change.
- ▶ MI clinicians therefore **seek to increase their patients ' optimism and confidence that they can change.**



Process of motivational Interviewing



OARS: Basic tools for building motivation to change

Open questions

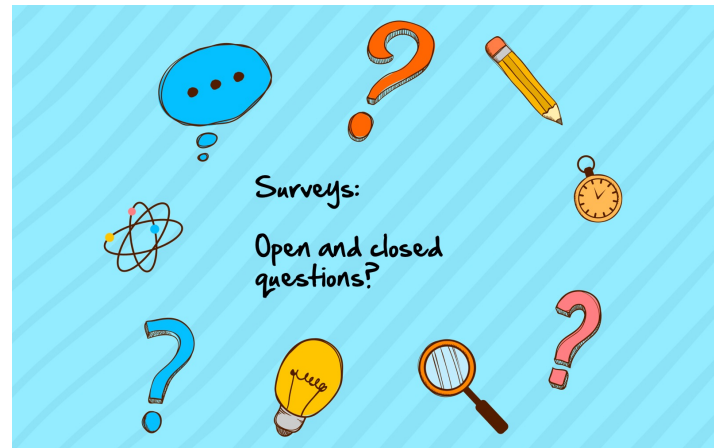
Affirmations

Reflective listening

Summarizing

Use open ended questions

- ▶ Open questions lead the patient to talk more, providing more of a picture of the situation as he or she sees it.
- ▶ General rule is to **avoid** asking **three or more questions consecutively**.



Affirm patients

- ▶ Affirming patients can **enhance rapport** and **help support patients' confidence**.



Use reflective listening

- It is a method of **responding to a patient** in which the **clinician repeats** or **paraphrases** what **he or she** has **heard the patient say**.



Use summaries

- ▶ To **link information** that has been presented **by the patient** in few short sentences.

SUMMARY

Elicit change talk

- ▶ Statements made by a patient that are “**in the direction of change**” are termed “change talk.”

Responding to change talk

- ▶ Once **change talk** has been **elicited**, the goal in MI is to **reinforce and encourage more change talk**.

100

- [illegible]



Thank you